ALL INDIA INSTITUTE OF MEDICAL SCIENCES, RISHIKESH
DEPARTMENT OF GENERAL SURGERY
(UNIT-II)
DISCHARGE SUMMARY

NAME		**AGE/SEX**	59 years / Male
UHID		**DOA**	06/10/26
ADDRESS		**DOP**	08/10/26
Phone No.		**DOD**	
DIAGNOSIS	Left Thigh Intramuscular Lipoma		
MANAGEMENT	Lipoma excision		
UNIT II		**CONSULTANT IN-CHARGE**	

CHIEF COMPLAINS: Swelling over medial aspect of left thigh since 1 year
HISTORY OF PRESENTING ILLNESS:. The patient was apparently normal 1 year back when he noticed a swelling over the medial aspect of the left thigh, which was insidious in onset, gradually progressive, and not associated with pain or impairment of function. There is no history of fever, cough, melena, trauma, sinus, discharge, hair growth over mass, or skin changes.
Past medical history: H/O HTN since 4 years, not on regular medications, now started on T. Telma AV 40/5 OD. No DM, IHD, COPD, Asthma, TB.
Past surgical history: No past surgical history
Personal History: Married, pure vegetarian diet, normal sleep, adequate appetite, normal bowel and bladder habits.
Menstrual History:
Addiction: Reformed smoker since 20 years of age, 5 bidis/day (last smoked 20 years back)
Drug Allergies: No known allergies
Family history: Not significant
Treatment history: Tablet Telma AV 40/5 mg OD
Examination (at admission):

GC	Conscious, well oriented to time, place & person, alert and cooperative. No pallor, icterus, clubbing, cyanosis, edema or lymphadenopathy.		
PR	90/min	**SPO2**	98% on RA
BP	134/93 mmHg	**Height**	167 cm
RR	22/min	**Weight**	61 kg
Temp	Afebrile	**BMI**	21.9 kg/m2
CNS	No focal neurological deficit, meningeal signs absent, cranial nerves intact, motor power 5/5 in all four limbs, sensory system intact.		
R/S	Trachea central, bilateral chest rise present, bilateral normal vesicular breath sounds present, no added sounds.		
CVS	Precordium normal, apex beat at 5th intercostal space at mid-clavicular line, S1 S2 heard, no murmurs.		

P/A:
Inspection: Umbilicus central and inverted. All quadrants move proportionally with respiration. No fistula, no dilated veins. All hernial orifices intact.
Palpation: No local rise of temperature or tenderness. No organomegaly. No lump.
Percussion: Tympanic note present, no fluid thrill or shifting dullness.
Auscultation: Bowel sounds present.
DRE:

External: No external hemorrhoids, sinus, fissures, fistula.
Internal: Anal tone normal, intact mucosa, no growth, fecal staining present.

INVESTIGATIONS:

	06/10	09/10
HB	12.6	12.7
TLC	6.38k	8.70k
PLATELET	258k	257k
UREA/Cr	44/0.77	
Na/K/Ca	142/4.3/8.9	

Total BIL.	0.23	
Direct Bil.	0.1	
SGPT/SGOT	25/17	
Total Protein	7	
Albumin	4	
ALP/GGT	78/24	
PT/INR	11.8/0.69	
VIRAL MARKER	HBV: (NR) / HCV: (NR) / HIV: (NR)	
HBA1C	6.2	

RADIOLOGICAL INVESTIGATIONS	
MRI Left Thigh (19/08/2026)	Clinical history: swelling in left thigh for 1.5 years with gradual increase in size, FNAC s/o spindle cell lipoma Study Procedure: Multi planar images of right thigh were obtained using T1, T2, PD fat sat & STIR sequences. T1 and PD fat sat sequences were repeated after administration of IV contrast. IMAGING FINDINGS: • A well defined fat density lobulated lesion with few T2 hypointense septa ~ 13.3 x 6.5 x 10.6 cm is seen in the intra and intermuscular plane of the medial group of left thigh. It is seen displacing adductor magnus laterally , medially extending along the ischiofemoral space superiorly abutting the obturator internus, laterally along the hip capsule and gemelli posteriorly. It is also seen abutting the ischium. The lesion shows T1/T2 hyperintensity with suppression on FAT-SAT images. No enhancing solid component or diffusion restriction is seen within. Intraiesional septa show mild enhancement. No intra pelvic extension or extension to hip joint. No edema or abnormal enhancement in surrounding muscles. No underlying bone marrow edema in femur. • Rest of the thigh muscles in the entire compartment appear normal in signal intensity and bulk. No evidence of abnormal signal intensity is seen. • Visualized femur shows normal signal intensity. • Neurovascular bundles reveal no abnormality. • No evidence of any signal intensity lesion seen in the subcutaneous plane. • No evidence of any other abnormally enhancing lesion in visualized thigh on post contrast study. • Few subchondral cysts are seen in the head of femur - degenerative. Impression: Features suggest left thigh intramuscular lipoma.
CECT Thorax (24/08/2026)	Clinical information: K/C/O soft tissue sarcoma of left medial upper thigh. To look for lung metastasis. H/O smoking present for 20 years. Study Procedure: A plain and post contrast CT study of the thorax is performed using contiguous sections from thoracic inlet to thoracic outlet. Report was made after reviewing all the DICOM images at the workstation. IMAGING FINDINGS: • RB-ILD changes seen in bilateral lung parenchyma. • Bilateral basal dependent densities seen. • A small cyst measuring ~5.7 x 5.8mm seen in the postero-basal segment of the right lower lobe. • Few tiny subcentimetric nodules seen in the bilateral upper lobes- too small to characterize.

	• Few subcentimetric perifissural nodules seen along bilateral oblique fissures largest measuring ~5.7 x 3.2mm along the right oblique fissure. • There is no pleural or mediastinal mass. • No bilateral pleural effusion noted. • The anatomical configuration of the structures in the mediastinum and both hila appear normal. • Trachea and mainstem bronchi appear normal. • The mediastinal vascular structures are normal. • Note is made of few subcentimetric mediastinal lymph nodes, largest ~ 7.6 mm in sub-aortic station. • Visualised portion of esophagus does not show any mass lesion or wall thickening. • Both diaphragms are normal and no evidence of sub-diaphragmatic pathology is seen. • Visualised upper abdominal structures do not show any significant abnormality. • Visualised vertebrae show diffuse osteopenia. • Cardiothoracic ratio is 0.48 IMPRESSION: -RB-ILD changes with bilateral basal dependent densities in bilateral lungs. - No evidence of thoracic metastasis in present scan.
HPE (02/09/2026)	Specimen sent: USG guided biopsy from left thigh lesion Clinical Diagnosis: Left thigh lipoma Gross: Received multiple grey white linear soft tissue cores ranging in length from 0.2 to 0.8 cm. All embedded as A1. Report: Section shows adipose tissue only. No evidence of malignancy identified in section examined. Impression: Features are of lipomatous lesion.

Procedure	Lipoma excision under spinal anesthesia on 08/10/2026
Course in hospital	The patient was admitted with the above-mentioned complaints. Relevant investigations were done and he was diagnosed as a case of Left Thigh Intramuscular Lipoma. He underwent Lipoma Excision under spinal anesthesia on 08/10/2026. Intraoperatively, a large intramuscular lipoma measuring 15 cm x 10 cm was identified in the left thigh within the adductor magnus muscle, extending up to the ischiofemoral space medially and the ischial tuberosity laterally. The swelling was excised in toto, haemostasis was achieved, and a 16Fr Romovac drain was placed in the cavity. Postoperative period was uneventful. Currently, the patient is hemodynamically stable, accepting orally, ambulatory, passing flatus and feces, with a healthy surgical wound and is being discharged with the following advice.
Condition at discharge	Currently, the patient is hemodynamically stable, accepting orally, ambulatory, passing flatus and feces, with a healthy surgical wound and is being discharged with the following advice.
Advice at discharge	1. Maintain personal hygiene. 2. Tab Ceftum 500mg PO BD x 5 days. 3. Tab Pantop 40mg PO OD x 5 days. 4. Tab PCM 650mg PO SOS (if pain/fever). 5. Tab Emeset 4mg PO SOS (if nausea/vomiting). 6. Chest physiotherapy to continue. 7. Incentive spirometry to continue. 8. Steam inhalation to continue. 9. Tablet Telma AV 40/5 mg OD to continue. 10. Tablet Telvas AM 40/5 mg PO OD 11. Tablet Pantop 40 mg PO OD 12. Tablet PCM 500 mg PO SOS 13. Tablet Diclofenac 75 mg PO SOS 14. Tablet Emeset 4 mg PO SOS 15. Wound care and dressing advice as per protocol

FOLLOW UP	1. Follow up in General Surgery OPD Unit II (Tuesday/Friday) or in case of emergency contact 01352462503. 2. Follow up in General Surgery OPD after 7 days with HPE report.

JUNIOR RESIDENT	SENIOR RESIDENT	CONSULTANT
[redacted]	[redacted]	[redacted]